STIVEN CATALYST

SECOND SERIES · NOVEMBER 2026

MANAGEMENT REVIEW

Management without theatre.



PEOPLE

JUST CULTURE: ACCOUNTABILITY WITHOUT FEAR

Reason's line between blameless and blameworthy, how many NHS staff think people involved in errors are treated fairly, Marx's three behaviours, a trust that moved from blame to repair, who draws the line, and a card for the decision after an error.

59% of NHS staff in England say their organisation treats staff involved in an error, near miss or incident fairly.

NHS STAFF SURVEY, 2025

INSIDE

COVER STORY
Where the line is drawn
PAGE 03

THE MODEL
Three behaviours, three answers
PAGE 05

TOOL OF THE ISSUE
The card for after an error
PAGE 08

NO. 50 · PEOPLE

In this issue

When something goes wrong, a team quickly learns what is safer: to speak up or to keep quiet. A clear line between an honest mistake and behaviour that deserves a consequence lets people keep reporting and still be held to account.

James Reason saw that blaming a person is more satisfying than fixing a system, and wanted a shared line between blameless and blameworthy acts. In the 2025 NHS Staff Survey, 86% of staff said they were encouraged to report errors, but only 59% that those involved were treated fairly. David Marx separates human error, at-risk behaviour and recklessness. A trust in Liverpool replaced culpability with repair, and Sidney Dekker asks who gets to draw the line.

Stiven Janaqi EDITOR · STIVEN CATALYST

03 COVER STORY
Where the line is drawn

04 THE NUMBERS
Encouraged to report, not sure it is safe

05 THE MODEL
Three behaviours, three answers

06 WHAT THE RESEARCH SAYS
Not who is to blame, but who was hurt

07 HOW IT IS MEASURED
Who draws the line

08 TOOL OF THE ISSUE
The card for after an error

09 SOURCES
Sources and method

HOW TO READ THIS ISSUE

Figure
Every figure has its source and year at the foot of its page.

Our reading
Where the editors interpret rather than the research, it says so.

Practice
The steps and the card are proposals to try, not research results.

COVER STORY

Where the line is drawn

In 2000 the psychologist James Reason described two ways of looking at human error in the BMJ. The person approach looks for the forgetful, inattentive or careless individual. The system approach treats errors as consequences rather than causes, and asks how and why the defences failed.

The person approach	The system approach
Who made the mistake?	How and why did the defences fail?
Posters, retraining, discipline	Change the conditions of the work
Naming, blaming and shaming	Find the weak spots before they line up

Reason knew which one comes easier: “Blaming individuals is emotionally more satisfying than targeting institutions.” Yet learning needs reports, reports need trust, and trust needs a shared understanding of where the line runs between blameless and blameworthy acts. In aviation maintenance, he noted, some 90% of quality lapses were judged blameless.

OUR READING

A rule about blame is also a rule about information: whatever gets punished stops being reported.

Sources: James Reason, BMJ 320(7237), 2000; James Reason, Ashgate, 1997 (via ISMP Canada, Hospital News, April 2007; ANA, 2010)

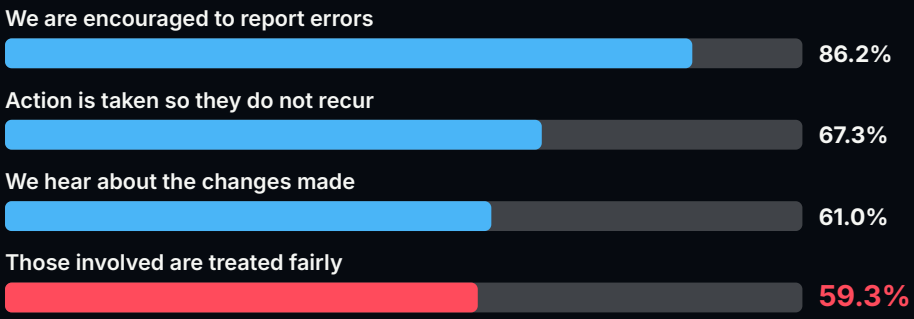
An essay that reports no new study; the 90% comes from a 1997 paper by David Marx that we did not see. Reason's book of 1997 already called a just culture an atmosphere of trust with a clear line.

THE NUMBERS

Encouraged to report, not sure it is safe

Every year the NHS Staff Survey asks the staff of all trusts in England about errors, near misses and incidents. In autumn 2025, 766,285 people answered, a response rate of 49%. The share who agree:

NHS STAFF IN ENGLAND WHO AGREE, 2025



A third of staff (33.7%) had seen an error, near miss or incident in the last month that could have hurt staff or patients. The share who say those involved are treated fairly has hardly moved: 58.2% in 2022, 59.3% in 2025.

OUR READING

Asking for reports is the easy step. Whether people keep reporting depends on what happens next, to the person and to the problem.

Source: Survey Coordination Centre for NHS England, 2026

"Agree" or "strongly agree", weighted results for the 206 NHS trusts. Self-reports from one health system; for Albania and for other sectors we found no comparable figure.

THE MODEL

Three behaviours, three answers

In 2001 David Marx wrote a primer on just culture for hospital executives, from a lawyer's point of view. Judge the behaviour and the risk it carried, he argues, more than the outcome. In his later model, as the American Nurses Association summed it up in 2010, each kind of behaviour calls for a different answer.

01

Human error: console

unintentionally doing other than what should have been done; then look at process, training and design

02

At-risk: coach

a choice whose risk was not seen, or was wrongly thought justified; remove the reward for the shortcut

03

Reckless: sanction

conscious disregard of a substantial and unjustifiable risk; remedial or punitive action

EU law draws a similar line for aviation. Regulation 376/2014, applied since November 2015, defines just culture as one where front-line staff are not punished for actions, omissions or decisions in line with their experience and training, but where gross negligence, wilful violations and destructive acts are not tolerated.

OUR READING

Most hard cases sit in the middle box: the shortcut everyone takes because the official way is slower.

Sources: David Marx, Trustees of Columbia University, 2001 (via The three behaviours: ANA position statement, 2010); European Parliament and Council of the EU, 2014

The three behaviours and answers follow the ANA's summary of Marx's later work; his 2001 primer names four: human error, negligence, recklessness, knowing violations. The reading is the editors'.

More in the essay How to talk to someone who made a mistake

WHAT THE RESEARCH SAYS

Not who is to blame, but who was hurt

In 2016 Mersey Care, an NHS mental health and community trust in the Liverpool region with about 8,000 staff, changed how it responds to incidents. Instead of asking how culpable an error was and what consequence fits, it asked: who is hurt, what do they need, and whose obligation is it to meet those needs?

DISCIPLINARY AND SUSPENSION CASES, TWO OPERATIONAL UNITS



Over the same years, reports of adverse events rose by 7% to 18% a year, and requests for face-to-face counselling went from an average of 283 to 378 a year. The authors put the economic benefit at about £2.5 million, around 1% of total costs, after crediting only half of the savings they found to the new culture.

OUR READING

Fewer cases alone proves little. More reports together with fewer suspensions is the pattern worth looking for.

Source: Mannat Kaur, Robert J. de Boer, Amanda Oates, Joe Rafferty & Sidney Dekker, MATEC Web of Conferences 273, 2019

One trust, before and after (April 2014 to March 2018), with no benchmark to check against, as the authors note. Two of the five authors worked at Mersey Care.

HOW IT IS MEASURED

Who draws the line

Sidney Dekker's Just Culture (2007) asks when an honest mistake stops being honest, and then moves the question. In the words of the book's description:

> “What matters is not where the line goes, but who gets to draw it.”

1. **We are encouraged to report**
   Count reports per month, near misses included.
2. **Those involved are treated fairly**
   Note what happened to the person after each report.
3. **Action is taken**
   Count the reports closed with a change rather than just a reply.
4. **We hear what changed**
   Days from the report to feedback for the person who made it.

HYPOTHETICAL EXAMPLE, A QUARTER IN A PARCEL DEPOT

Reports	31, 12 of them near misses
Changes	9 closed with a change
Feedback	on 5 of the 9, after 19 days on average

The reports came; the answers were slow. Depot and figures are invented.

Sources: Sidney Dekker, Ashgate, 2007 (via Publisher's description); Survey Coordination Centre for NHS England, 2026

The four statements follow questions 19a–d of the NHS Staff Survey; the counts and the example are the editors'. Dekker's sentence is from the publisher's description; the translations are ours.

TOOL OF THE ISSUE

The card for after an error

One card per person involved, filled in before anyone decides on a consequence. The first lines cover the work and the system, only the last the person.

01 What happened facts, times and places; no labels

02 Intended or impaired? harm meant, or signs of illness or substances? Then another procedure applies

03 The rule was there a procedure; was it available, workable and in routine use?

04 A peer in the same place would someone with the same training and experience have done the same?

05 Behaviour and answer human error: console · at-risk: coach · reckless: sanction

06 System change and feedback what changes, who owns it, by when; when the reporter hears

Sources: National Patient Safety Agency (England), 2003 (via Flowchart reproduced in an NHS trust's investigation guidelines, Thirlwall Inquiry evidence INQ0003324); David Marx, Trustees of Columbia University, 2001 (via The three behaviours: ANA position statement, 2010)

The card is the editors' proposal, after the four tests of the NPSA Incident Decision Tree, which rests on Reason's culpability model, and Marx's three behaviours. It replaces neither an investigation nor HR procedures.

Open the tool: 5 Whys stivencatalyst.com/tools/five-whys/

SOURCES AND METHOD

Every figure has a source.

The figures in this issue come from the sources below. The year shows how recent each one is.

Human error: models and management
James Reason, BMJ 320(7237), 2000
https://doi.org/10.1136/bmj.320.7237.768

Managing the Risks of Organizational Accidents
James Reason, Ashgate, 1997 · via ISMP Canada, Hospital News, April 2007; ANA, 2010
https://www.ismp-canada.org/download/hnews/HNews0704.pdf

2025 NHS Staff Survey: National results briefing
Survey Coordination Centre for NHS England, 2026
https://www.nhsstaffsurveys.com/results/national-results/

Patient Safety and the "Just Culture": A Primer for Health Care Executives
David Marx, Trustees of Columbia University, 2001 · via The three behaviours: ANA position statement, 2010
https://psnet.ahrq.gov/issue/patient-safety-and-just-culture-primer-health-care-executives

Regulation (EU) No 376/2014 on the reporting, analysis and follow-up of occurrences in civil aviation
European Parliament and Council of the EU, 2014
https://eur-lex.europa.eu/eli/reg/2014/376/oj

Restorative Just Culture: a Study of the Practical and Economic Effects of Implementing Restorative Justice in an NHS Trust
Mannat Kaur, Robert J. de Boer, Amanda Oates, Joe Rafferty & Sidney Dekker, MATEC Web of Conferences 273, 2019
https://doi.org/10.1051/matecconf/201927301007

Just Culture: Balancing Safety and Accountability
Sidney Dekker, Ashgate, 2007 · via Publisher's description

Incident Decision Tree
National Patient Safety Agency (England), 2003 · via Flowchart reproduced in an NHS trust's investigation guidelines, Thirlwall Inquiry evidence INQ0003324
https://thirlwall.public-inquiry.uk/wp-content/uploads/thirlwall-evidence/INQ0003324_15-16.pdf

EDITORIAL METHOD

Each figure was checked for its year, its publisher and what exactly it measures. Where the publisher's page could not be opened, the figure was checked against independent summaries and is marked "via". The editors' interpretation is marked "Our reading". Figures that could not be confirmed are not in the issue.



MANAGEMENT REVIEW

Management without theatre.

Every issue, one management question, checked against the best research.

ALL ISSUES

stivencatalyst.com/magazine/management-review.html